



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : تغريد مشكور عبدالعزيز محمد

المحافظة : القاهرة الحي / المدينة : مدينه السلام

رقم البطاقة : 27608010105544 رقم التليفون : 01223782863

العنوان : السلام تاريخ تسجيل الإستمارة : 21-09-2026

العمر (تاريخ الميلاد) : 01-08-1976 المهنة : حاصل علي بكالوريوس

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج بعقار TDM1 وريدي [14121] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: تغريد مشكور عبدالعزيز محمد National ID: 27608010105544	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي IV (trastuzumab emtansine) TDM1
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٢٩٧

Patient Name : الأستاذ / تغريد مشكور عبد العزيز محمد
Patient No. : 27608010105544
Path No. : 515/26
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13005166
Request Date : 23/02/2026
Verification Date : 02/03/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Clinical Data : Right breast cancer (Post NACT).

Nature of the specimen : Right wide local excision & right sentinel lymph nodes biopsy

Gross Examination:

- Procedure: Right wide local excision specimen measured 6x5x4 cm , No skin covering .
- Tumor extent : A fairly defined greyish white tumor bed site measured 5x2x1cm .
- Distance from margins: 0.3 cm away from lateral margin .
- Lymph nodes number and size: A fibrofatty tissue piece measured 5x3x1 cm enclosing 5 SLNs up to 1.5x1 cm .

-Clipped intramammary lymph node measuring 1x1 cm also received separately

• Frozen section:

-SLNs:Free lymph nodes (0/5).

-Intramammary lymph node :Free intramammary lymph node

-Margins :Free margins.

*Blocks:

1.medial 2.lateral 3.superior , 4.Inferior , 5. superficial 6.Deep 7.SLNs, 8.Intramammary lymph node, 9:13 TBS.

Grossing done by :Dr.Heba El-Shafeey.

Microscopic examination:

- Histological type: A tiny focus of residual Invasive mucinous carcinoma, cellularity 2 %
- Grade :(tubular differentiation 2, nuclear pleomorphism 2,mitosis 2):2
- Angioinvasion:Absent.
- Ductal carcinoma in situ: Absent.
- Lymph node involvement: Free seven lymph nodes of metastatic deposits. (0/7) including intramammary lymph node.
- Resection Margins: Free margins.

Diagnosis:

Verified by :

Dr.Heba Hasan

Printing Date : 04/03/2026 12:37:19

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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Pathology Report

Right wide local excision & Right Sentinel lymph nodes biopsy:
Tiny focus (0.2ml) of Invasive mucinous carcinoma .
• Lymph nodes :Free LNs (0/7), including free intramammary lymph node
• Margins :Free margins.
• Stage : pT1a , pN0
-RCB : 1.374 RCB class :II, CELLULARITY 2 %

Prof .Dr.Maha Guimei
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 04/03/2026 12:37:19

Page 2 of 2

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جمهورية مصر العربية
 وزارة الداخلية
 إدارة الأحوال المدنية
 بطاقة هوية
 رقم البطاقة: ٢٧٦٠٨٠١١٠٥٥٤٤
 تاريخ الميلاد: ٢٠١١/٠٨/٢٧
 الاسم: محمد عبد العزيز محمد
 رقم البطاقة: ٢٧٦٠٨٠١١٠٥٥٤٤
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٢٠١١/٠٨/٢٧

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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالمنشأة المستهدفة
صحة

03/17/2026

MDT FORM

Name:	Taghrid Mashkor Abdelaziz.	MRN:	2297
Age:	50 Y	ID:	27608010105544
Menopausal Sta	pre	Phone:	01223782863
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	free	Reason Of MDT:	post op

Clinical Data:

BX:	Invasive duct cardnema with lobular cancerization						
IS:	8/8	PR:	8/8	HER2:	3+	K167:	35

Sonomammography:

Mets Workup:

History:

Right wide local excision & Right Sentinel lymph nodes biopsy: POST NACT
Tiny focus (0.2ml) of Invasive mucinous carcinoma.

• Lymph nodes :Free LNs (0/7), including free intramammary lymph node

• Margins :Free margins .

Stage:pT1a,PNO

for possibility of adj ttt

omnia essawy

Recommendations:

MDT Outcome & Plan

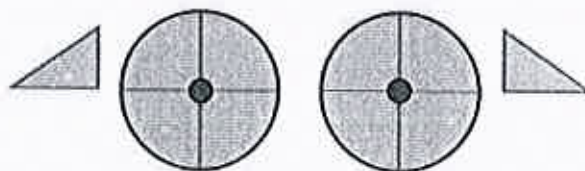
To continue Phesgo up to 1 year + Femara + Zoladex

PORT

Zometa / 6m

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد التيرابي				

Histopathology Report

Patient Name :	الأستاذة / نوريد مشكور عبد العزيز محمد	Request No. :	13000857
Patient No. :	27608010105544	Request Date :	14/06/2025
Path No. :	2028/25	Verification Date :	19/06/2025
Sex :	Female	Financial Category :	بلدية الدولة
Age :	48 Y	Department :	Out Patient
Referring Doc :		Registered by :	Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass

Nature of specimen

Tru cut biopsy, collected at 12/6/2025, 10:30 am

Gross examination

Multiple(4) grayish white tissue cores up to 1 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma with lobular cancerization
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade 2
- * DCIS: Focal, necrosis
- * Angiolinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
- * Ki 67: 35% nuclear reaction of tumor cells

Diagnosis

Right breast mass, Tru cut biopsy:

Verified by :

Dr. Heba Hasan

Printing Date : 29/06/2025 11:55:39

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Histopathology Report

Patient Name : الأستاذة / نغريد مشكور عبد العزيز محمد
Patient No. : 27608010105544
Path No. : 2028/25

Sex : Female
Age : 48 Y
Referring Doc :

Request No. :
Request Date : 13000857
Verification Date : 14/06/2025
19/06/2025
Financial Category : فئة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Invasive duct carcinoma, Grade 2 with lobular cancerization

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: Positive (score 3+).
- * Ki67: Positive in 35% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim
Dr. Heba Hassan Adly

Verified by :

Dr. Heba Hasan

Printing Date : 29/06/2025 11:55:39

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Name: Mrs. Taghreed Mashkour Abdulaziez

Date: 2025-12-30

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven Right breast cancer, post NACT assessment study.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UOQ, posterior third, fat containing circumscribed lesion with tissue marker within and upper central focal asymmetry with tissue marker within and surrounding distortion.
- **Ultrasound** demonstrated 12 o'clock, middle third, 6 mm deep to the skin, area of granulation tissue with negative vascularity within measuring 24 x 7 mm with surrounding distortion and tissue marker within.
- 10 o'clock outer third intra-mammary lymph node measuring 8 x 6 mm with cortical thickness of 2.5 mm showing tissue marker within.
- Normal nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Near total radiological response of the previously reported malignant process.

Left breast (BIRADS 2): diffuse breast adenosis.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Patient ID: 86803	Patient Name English: Taghrid Mashkor Abd Alaziz Mohamed
Age: ٢٩ سنة	2025/08/18 11:31:51 AM :Report Date

Women's Health Outreach Program

Name: Taghreed Mashkoor Abdel Aziz

Date: 18/08/2025

Ultrasound Guided Clip Insertion

Indication: Known case of right breast cancer (IDC grad2 with lobular cancerization , +ve ER, +PR and Positive Her2 an Ki67= 35%)

Right breast

- Under complete aseptic condition & ultrasound guidance two clip insertion were done at the centre of malignant mass lesion and intramammary node as the following:
 - **Clip (1)** : Inserted at the lesion located at 12 o'clock position and measures about 30x35mm ,
 - **Clip (2)** : Inserted into the intramammary lymph node located at 10 o'clock position
- They are confirmed by mammographic CC & ML

*Best Regards,
Dr. Marwa M. Hassan.*

Name : Taghreed Mashkoor Abdulaziz
Mohammed

Date : 18-10-2025 10:58

Branch : Dokki

Medical No. : 227687

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 48-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

GUSTAVE/
ROUSSY/
INTERNATIONAL

جوستافي
روسي
الترافيق
1339

ECHO REPORT

NAME :

فريد عبد العزيز

DATE:

18 / 8 / 2022

ID NUMBER : 1471/0

SEX :

AGE:

EF	64%	55-75%	PWT	0.6	0.6-1.1cm
LVEDD	3.6	3.5-5.8cm	AO.R	1.0	3-3.7cm
LVEDS	35.3%	2-4cm	LA	3.5	2-4cm
FS	33%	25-45%	RV	2.0	0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP	—	1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

Patient's Name:

Taghreed Mashkoor Abdulaziz Mohammed

Date:

2026-08-18

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.

Dr.Asser Ahmed

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Dr.Asser Ahmed

GUSTAVE/
ROUSSY/
INTERNATIONAL

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Opinion:

- ❖ No hepatic or pulmonary metastatic lesions.

Dr.Asser Ahmed

$$\sqrt{\frac{1}{2\pi\sigma^2}}$$

بشأن تقرير طبي للمريض:
.....

تاريخ وساعة الخروج.

وبعد الكشف والفحوصات والمتابعة وجد انه:

Colecta husband + wife + 10 relatives
contact a Phage then under strict R+ will

for Dingo but refused from 17 to 19 miles

new pickup truck due in 8/11/16

الشيخ :: / /

So far everything.

بیعت

Page 1 of 1